Module # 3

Relapse Prevention
Part 1

By

Bill Urell

Addiction Recovery Basics

http://addictionrecoverybasics.com/

LEGAL DISCLAIMER: Disclaimer Last Updated: February 24, 2008 DISCLAIMER: This information is not presented by a medical practitioner, and is for educational and informational purposes only. The content is not intended to be a substitute for professional medical advice, diagnosis, or treatment. Always seek the advice of your physician or other qualified health provider with any questions you may have regarding a medical condition. Never disregard professional medical advice or delay in seeking it because of something you have read.

Introduction

In this part of the course we're going to take a look at a working definition of the relapse process. Hopefully, we will be able to increase our awareness of the early signs of relapse.

When these early warning signs are recognized, it becomes possible to stop the process in its infancy and minimize the chances of having a using episode. The work done now, looking for early warning signs, recognizing your own triggers and cues, and constructing prevention plans to deal with them, can pay major dividends when crisis occurs.

It is our hope that over the next few weeks, you will come to have a thorough understanding of the relapse process and have developed a personalized plan to help you deal with any of the warning signs or dangerous signals that may arise.

Onward,

Bill Urell

© 2008 Bill Urell

Relapse Part 1

Addiction is known as a chronic relapsing disorder. Since relapse is such an integral part of addiction and addiction recovery, it makes sense that the serious business of preventing relapse is an absolutely critical element in alcohol and drug treatment.

According to the Department of Health and Human Services, Technical Assistance Publication No. 11:

> "Addiction is a chronic relapsing disorder, thereby making the prevention of relapse one of the critical elements of effective treatment for alcohol and other drug (AOD) abuse. Studies have shown that 54 percent of all alcohol and other drug abuse patients can be expected to relapse, and that 61 percent of that number, will have multiple periods of relapse. It is not unusual for addicts to relapse within one month following treatment, nor is it unusual for addicts to relapse 12 months after treatment; 47 percent will relapse within the first year after treatment (Simpson, Joe & Lehman 1986). **Although relapse is a symptom of addiction, it is preventable."**

Now, *I* put that emphasis on the last sentence of the quote above. One of my pet peeves is misinformation about the subject of relapse. It never fails that some well-intentioned soul will say something like "Only 5 percent of the people who give up drugs and alcohol are successful", or, something like an "Addiction professional" saying to a packed lecture hall in rehab, "look to the left of you and look to the right to view those people won't make it." That happened to me. In my opinion, not only is that a disheartening thing to say, but it is simply not true.

Simply because one has a relapse certainly does not mean that they will never "get it". You are not necessarily doomed to everlasting drug and alcohol use because of the occurrence of a relapse. Another study shows:

> "Studies of lifelong patterns of recovery and relapse indicate that ***approximately one-third of patients achieve permanent abstinence through their first serious attempt at recovery.***

© 2008 Bill Urell

> Another third have brief relapse episodes which eventually result in long-term abstinence. An additional one-third have chronic relapses which result in eventual recovery from chemical addiction (Gorski, Kelley & Havens, 1993).

Now I see those statistics as being quite a bit different from only 5% make it.I'm sorry if I got off on a bit of rant there, but since I'm writing this I guess it's allowed.

The Relapse Process

It is very important to understand that relapse, just like recovery, is a process not an event. In fact when we discuss the relapse process, it actually ends when a person picks up a drink or drug. At that point you are entering a using episode. Everything that happens before picking up is actually the relapse process.

When, exactly, somebody enters in to the relapse mode is very difficult to define with precision. What happens is that very subtle changes occur in a person's attitudes, beliefs, and emotions, all leading to the point where return to chemical use makes sense. It is these subtle changes in attitudes and thought processes that move a person from "I know I can't use >>>>>>>>> I can use once more and control it."

Let's take all look a four stage model of relapse:

4 Stages of Relapse

First stage - I am unaware. I don't see it, and have no idea that I am in trouble. (People around us may notice subtle changes in attitude and behavior).

Second Stage - We become restless, irritable and discontent. Our focus shifts from internal to external; we stop focusing on ourselves and start focusing on other people around us. We start blaming and acting the victim. Fear and anger start to become evident.

© 2008 Bill Urell

Third stage - Unresolved feelings occur and they are not dealt with in a healthy manner. We go into emotional and physical withdrawal. We start to isolate. Negative attitudes such as compulsive behavior start to predominate. We start discounting recovery, we engage magical thinking**.**

Fourth stage - A crisis in our life causes or provides the excuse for us to start using again, or, we create a crisis that rationalizes our return to use. In other words, we have made the decision to use, and are ready to light the fuse.

As we move through the stages of relapse, a few different things are occurring. Firstly, the need to regain our 'right' to re-engage in our addiction seems to make sense. We talk ourselves into the false belief that this time we can control it.

Secondly, there is a gradual and progressive destabilization of our lifestyle. Lastly, Stress and Stressors will accelerate this process. The stress factors we're talking about here can best be pointed out by observing the warning signs of relapse.

Contributing Factors to Relapse:

It is really important to understand some of the personal factors that could possibly contribute to the relapse process and end up in a return to drug or alcohol use. Here are 5 factors:

1. Exposure to any person, place, feeling, or situation where we have used drugs, alcohol, or experienced emotional trauma.

2. Frequent exposure to high-risk situations that have led to drug or alcohol use in past.

3. Recurrent thoughts or physical desires (cravings), to use drugs or alcohol.

© 2008 Bill Urell

4. Inadequate living skills to deal with personal conflict or negative emotions.

5. Inadequate skills to deal with social pressure to use substances.

Entering into addiction recovery requires changes in the aforementioned attitudes, behaviors and values. Relapse occurs when these attitudes and behaviors go back to ones similar to those exhibited when we were actively using alcohol or drugs.

Let's get very specific and identify some relapse warning signs.

Relapse Warning Signs:

I'm going to provide you with a pretty extensive list of warning signs, this is a case of if the shoe fits wear it, or take what you need and leave the rest. OK, here we go:

1. Getting impatient at that pace of which your recovery is happening.

2. Experimenting with controlled drinking or drug use.

3. Becoming overtired and fatigued.

4. Having the thought that it is OK to drink or drug if _____ (blank) occurs.

5. Making excuses for taking drugs or drinking; blaming it on people places and things

6. Starting to become jealous of people around you who can drink and drug in a controlled manner.

7. Not being ready for cravings.

8. Not expecting a bad days nor having a plan for them.

© 2008 Bill Urell

9. Expecting to get rewarded for staying sober. You should be doing this for your own sake.

10. Expecting people to believe your desire for sobriety too soon and at face value.

11. Blaming other people for your problems.

12. Not asking for help.

13. Stop going to aftercare or meetings.

14. Procrastinate on daily tasks.

15. Start to become defensive and let resentments build.

16. Start to associate with people who are drinking and using.

17. Start to become dishonest.

18. Start acting in a self important manner.

19. Begin having an "I don't care attitude".

20. Aspects of denial start to reemerge.

Knowing your own personal relapse warning signs can interrupt the relapse process and prevent your returning to drink or using drugs. Sometimes it's helpful to look back at the last time we started drinking or using after period of abstinence. If we look at this, it will provide us with our own personal relapse warning signs.

Identifying Personal Relapse Warning Signs

1. We had mentioned before that entering into relapse mode involves *some changes in thinking*. Perhaps the most dangerous change in thinking involves the

© 2008 Bill Urell

belief that you can once again pick up and start using, but control it. We also start remembering only the good times and not the bad times. The idea that we deserve it, or that drinking or using is a reward for our hard life becomes very attractive. We can also seduce ourselves into thinking that we just plain don't have the ability to stay sober.

List some specific examples about how your ***thinking*** changed before your last relapse, or your observation of another person's relapse:

1.

2.

3.

2. Not only does our thinking change but *our attitude changes* as we move closer and closer to relapse. Negative attitudes such as becoming self-centered, selfish, negative, and becoming lazy start to infiltrate our daily lives. And attitudes begin to develop that we're being asked to do too much, I mean we're only human and are being pushed to our limits as it is.

Try listing some very specific examples about how your **attitudes** have changed just before your last relapse or a relapse you might have observed and someone else:

1.

© 2008 Bill Urell

2.

3.

3. You might notice that in general, people engaged in good solid recovery behave a whole lot differently than people for using and abusing drugs and. These *behavioral patterns* are often easy to identify. Some typical signs that you may be sliding back into addictive behavior patterns are things like: neglecting personal hygiene, procrastinating, starting to take up high risk behavior is again, poor eating and sleeping habits, impulsivity, and showing a lack of self control.

List some specific examples about how your **behavior patterns** changed before your last relapse, or your observation of another person's relapse:

1.

2.

3.

4. Let' try to put it all together now. We have looked at the changes that occurred i*n your thinking, your attitudes, and your behavior patterns,* in that time before your last relapse, or one that you had observed in somebody else. Let's write them out. If you have never tried to stop using before and have never had an experience a relapse, just go ahead and was the patterns of behavior that were normal for you when you were using drugs or drinking either way list these warning signs of relapse, or patterns of using behavior.

© 2008 Bill Urell

1.

2.

3.

4.

5.

6.

7.

8.

9.

10.

Who said it was easy?

© 2008 Bill Urell

In Closing:

I hope you agree we did a pretty fair job at looking at the definition of the relapse process. It turned that it is a lot more to it than simply picking up a drink or a drug, in fact, as was previously illustrated; the return to use it is actually the end of the relapse process.

Chemical addiction is a disease, and, like many diseases, there is always the possibility of relapse. The process of alcohol and other drug abuse is complex, and is impacted by social, clinical, and medical factors. The solutions to the problem of chemical addiction are multi-faceted. Treatment strategies benefit from a relapse prevention component in virtually every case.

We hope you look forward to further investigations of relapse and relapse prevention over the course of the next two weeks.

In the meantime we wish you the best and happiest of recoveries.

Regards,

© 2008 Bill Urell